

PathScribe AI · ForMedrix

v0.9.0

PathScribe by ForMedrix AI

User Guide

Clinical Reporting System
For Pathologists and Residents

May 2026 · Confidential — PathScribe AI

Table of Contents

Table of Contents 2

1. Getting Started 4

 1.1 Logging In 4

 1.2 Biometric Setup 4

 1.3 The Navigation Bar 4

2. The Home Screen 5

3. The Worklist 6

 3.1 Status Tiles 6

 3.2 Table Columns 6

 3.3 Sorting 6

 3.4 Pool Cases 7

 3.5 Delegated Cases 7

4. Opening a Case 8

 4.1 Navigation within a Case 8

 4.2 Patient History 8

5. Diagnostic Information 9

6. Synoptic Reporting & AI Copilot 10

 6.1 AI Copilot Suggestions 10

 6.2 Accepting and Rejecting Suggestions 10

 6.3 Protocol Change Review (After Microscopic) 10

 6.4 Coding 10

7. Computational Sidecar 11

 7.1 Worklist Computational Icons 11

 7.2 Synoptic Report — Computational Tab 11

 7.3 Cancelling a Pending Order 11

 7.4 Ordering Additional Tests 11

8. Report Preview 12

9. Case Finalisation 13

 9.1 Pre-Finalisation Review 13

 9.2 After Finalisation 13

10. Case Delegation 14

- 10.1 Delegating a Case 14
- 10.2 Receiving a Delegated Case 14
- 11. Case Search 15**
 - 11.1 Smart Identifier Box 15
 - 11.2 Search Filters 15
- 12. Internal Messaging 16**
 - 12.1 Reading Messages 16
 - 12.2 Composing 16
 - 12.3 Request Informal Review 16
- 13. Voice Commands 17**
- 14. Your Profile 18**
- 15. Quick Reference 19**
- Appendix A — Colour & Visual Language Reference 20**
 - A.1 Case Status Indicators (Worklist dots) 20
 - A.2 Case Flags 20
 - A.3 TAT Progress Bars 20
 - A.4 Severity Badges (Quality Tab) 20

1. Getting Started

1.1 Logging In

Open Chrome or Edge and navigate to your PathScribe by ForMedrix AI address. Enter your email and password on the ForMedrix login screen and click **Sign In**. Credentials are issued by your system administrator.

1.2 Biometric Setup

On first login PathScribe may prompt you through the **Biometric Setup Wizard** (fingerprint or face recognition). You may skip this; it can be completed any time from your profile. Once enrolled, biometric sign-out replaces the password prompt in the Pre-Finalisation Review.

1.3 The Navigation Bar

Element	Action
PathScribe logo	Click anywhere to return to the Home screen.
Enhancement Request	Submit feature requests or bug reports.
Voice toggle	Enable/disable hands-free voice commands.
Messages	Open the internal messaging drawer.
Clinical Links	Quick access to CAP protocols, WHO Classification, and PathologyOutlines.
Your name / initials badge	Profile card — User Guide, Admin Guide (if admin), System Information.
Sign Out (arrow)	Log out of PathScribe.

2. The Home Screen

Tile	What it opens
Worklist	Your active case queue — the primary daily view.
Contribution	Personal productivity dashboard: workload, quality, and TAT metrics.
Search	Cross-institutional case search with advanced filters.
System Audit	Audit trail of all system events (admin-visible).
My Contribution	Detailed personal metrics by specialty, TAT trend, and quality.
Configuration	System settings, user management, template builder (admin role).

3. The Worklist

The Worklist is your primary clinical view. STAT / critical cases are pinned at the top under a red **Urgent** banner; remaining cases follow in sort order.

3.1 Status Tiles

Tile	Colour	What it shows
Total Cases	Grey	Your full non-pool caseload. Always a summary — not a filter.
Pool Cases	Indigo	Unassigned cases in the shared pool. Click a pool case to open the Pool Claim workflow.
Delegated to Me	Cyan	Cases routed to you by another pathologist for review or second opinion.
Critical	Red	STAT-priority or critically flagged cases.
In Progress	Blue	Cases actively being worked on in the reporting workflow.
Needs Review	Amber	Cases with an AI-generated synoptic draft awaiting your sign-off.
Amended	Purple	Finalised reports re-opened for amendment.
Completed Today	Green	Reports finalised today.

3.2 Table Columns

Column	Contents
● (dot)	Pulsing red dot = STAT/critical. No dot = routine.
Case	Accession number. Red = critical, cyan = routine. Click to open.
Patient	Last, First name (PHI). Sortable alphabetically.
MRN	Medical Record Number (PHI-protected).
Sex	M / F / O. Sortable.
DOB (Age)	MM/DD/YYYY with compact age label (e.g. 52y).
Specimen(s)	Up to 3 specimen label chips. Hover for full description.
Accession Date	Date specimen received in the laboratory. Sortable.
Physician	Requesting/submitting physician. Sortable.
Flag(s)	Coloured flag chips. Solid border = case-level. Dashed = specimen-level. Hover for name.
Status	Coloured dot: Draft · In Progress · Finalizing · Finalized · Amended.

3.3 Sorting

Click any column header to sort. Click again to reverse direction. PathScribe supports multi-column sort — up to three sort keys simultaneously. Sort preferences persist across sessions.

3.4 Pool Cases

Pool cases are unassigned cases visible to all pathologists at your institution. Click a pool case row to open the **Pool Claim** modal. Click **Claim** to assign it to yourself.

3.5 Delegated Cases

When another pathologist delegates a case to you, it appears in the **Delegated to Me** tile. Open the case and review the delegation note. You may accept the delegation or return the case with a comment.

4. Opening a Case

Click any worklist row to open the case. The case layout has three panels:

Panel	Contents
Left	Patient demographics, order details, clinical indication, requesting physician, and submitting client.
Centre	Diagnostic workspace: gross description, microscopic description, ancillary studies.
Right	Synoptic reporting form, AI Copilot suggestions, and the Computational Sidecar tab.

4.1 Navigation within a Case

Specimen tabs — multi-specimen cases show lettered tabs (A, B, C...) at the top of the right panel.

Breadcrumb navigation — click any crumb to return to that level. **Previous / Next Case** — use the arrow buttons in the case header bar (or voice commands).

4.2 Patient History

Click the **Patient History** link in the left panel to open the Patient History modal. This shows prior cases, relevant diagnoses, and clinical history for the patient. Dismiss with the close button, Esc, or click outside the modal.

5. Diagnostic Information

The centre panel contains three autosaving rich-text fields. The AI Copilot reads these in real time and generates synoptic suggestions as you type.

Field	Contents
Gross Description	Macroscopic findings — specimen dimensions, tumour measurements, margin distances, and macroscopic appearance.
Microscopic Description	Histological findings — tumour type, grade, invasion, margins, lymph nodes.
Ancillary Studies	IHC, FISH, molecular, flow cytometry, and any other supplementary results.

Tip: Enter the gross description first — the AI begins generating synoptic suggestions immediately. Complete or refine microscopic and ancillary text to trigger further suggestion updates.

6. Synoptic Reporting & AI Copilot

PathScribe uses CAP-compliant and RCPATH synoptic templates. The template displayed matches the specimen type and selected protocol.

6.1 AI Copilot Suggestions

Badge colour	Confidence	Meaning
Green	≥ 85%	High confidence — strongly supported by diagnostic text.
Amber	50–84%	Moderate confidence — review the source citation carefully.
Red	< 50%	Low confidence — treat as a starting point only; verify independently.

6.2 Accepting and Rejecting Suggestions

Action	How
Accept a suggestion	Click the tick (✓) icon — applied to the field immediately.
Reject a suggestion	Click the cross (✗) icon and enter your own value.
Accept all	Click Accept All at the top of the synoptic panel.

■ **Important:** AI suggestions are advisory only. You as the pathologist are responsible for reviewing and verifying every field before finalising. Confidence scores reflect computational certainty, not clinical accuracy.

6.3 Protocol Change Review (After Microscopic)

When you save a microscopic description, PathScribe AI re-evaluates whether the synoptic protocol assigned at gross is still appropriate.

NEW in v0.9.0

The Protocol Change Review modal shows each proposed change with:

- The current protocol and the proposed replacement side by side.
- The AI's clinical reasoning from the microscopic text.
- A confidence score for the proposal.
- A checkbox to accept or reject each individual change.

Select the changes you want to apply, or deselect any you want to keep as-is. Click **Apply Changes** to commit your selections. Click **Keep existing protocols** to dismiss without making changes. All decisions are recorded in the audit trail.

6.4 Coding

As synoptic fields are completed, PathScribe automatically suggests ICD-10 and SNOMED CT codes. Review and confirm, add manual codes, or override suggestions in the Coding panel.

7. Computational Sidecar

The Computational Sidecar brings live laboratory results — IHC, molecular, FISH, cytogenetics — directly into your PathScribe workflow.

7.1 Worklist Computational Icons

Icon colour	Status	What it means
Teal	Pending	Test ordered. No result received yet.
Amber	Preliminary	Interim result available. Final result still pending.
Green	Final	Final result received. No immediate clinical action required.
Red	Final — Actionable	Final result received. Review required — clinical action may be needed.

7.2 Synoptic Report — Computational Tab

Inside the Synoptic Report, click the **Computational** tab to open the full results panel. This shows specimen-grouped results with: result data table, status badge, specimen badge, AI concordance indicator, and AI extraction badge.

7.3 Cancelling a Pending Order

Click the trash icon beside a Pending test in the navigator. Read the confirmation dialog carefully — if the order originated in the LIS, you must also cancel it there.

7.4 Ordering Additional Tests

Click **+ Order additional test** at the bottom of the Computational navigator. Stage tests for one or more specimens, then click **Place N Orders** to submit.

Tip: You can stage tests for multiple specimens before submitting — change the Apply To selector between batches and accumulate all orders before clicking Place Orders.

8. Report Preview

Click **Preview Report** in the case header to open the formatted print-ready report.

Control	Function
Page size selector	Choose A4, US Letter, US Legal, A3, or B5. Your preference is saved.
Margin controls	Adjust top, right, bottom, left margins in mm. Saved per session.
Ruler	Shows margin boundaries above the document.
Print button	Sends the report to your browser print dialogue.

9. Case Finalisation

When all required synoptic fields are complete, click **Finalise**. Finalisation is a three-step gate:

Step	What happens
1 — Validation	PathScribe checks all required fields. Missing fields are highlighted. The gate will not advance until all required fields are filled.
2 — AI Triage Review	Fields the AI flagged as low-confidence (below the configured threshold) are shown one at a time. Review and confirm, override, or skip each item. Keyboard shortcuts: Space/→ = Confirm, O = Override, S = Skip, Esc = Cancel.
3 — Pre-Finalisation Review	Full-screen two-pane review before credentials are collected (see below).

9.1 Pre-Finalisation Review

NEW in v0.9.0

The Pre-Finalisation Review replaces the previous direct-to-password confirmation. It is mandatory for all sign-outs.

- **Left pane — Specimen list:** All specimens and their synoptic reports in transmission order. Drag specimens to reorder them. Click **Exclude** on any synoptic to remove it from this transmission (it remains in PathScribe as a draft for a future addendum).
- **Right pane — Report preview:** Live Q&A; formatted preview of the complete report output as it will be transmitted to the LIS. Updates in real time as you reorder or exclude synoptics.
- **Signing panel:** Sign using biometric (fingerprint or face, if enrolled) or your login password. This constitutes your clinical authorisation for the transmission and is recorded in the audit trail.

9.2 After Finalisation

Once finalised, the case status changes to **Finalized** and the report is transmitted to the LIS (Co-Pilot mode) or locked in PathScribe (Standalone mode). The report is locked for editing. Amendments require re-opening by an administrator.

■ **Important:** Resident counter-sign workflow: If you are a Resident, your report is submitted for attending review rather than directly finalised. The attending must counter-sign to complete the process.

10. Case Delegation

Delegation routes a case to a colleague for peer review, specialist opinion, second opinion, or subspecialty referral.

10.1 Delegating a Case

Step	Action
1	Open the case and click the Delegate button in the case header.
2	Select the recipient from the staff directory (search by name or subspecialty).
3	Choose the Delegation Type (Peer Review, Second Opinion, Subspecialty Referral, MDT Discussion).
4	Add a note explaining what you need from the recipient.
5	Click Confirm Delegation — the recipient receives a notification and the case appears in their Delegated to Me tile.

10.2 Receiving a Delegated Case

Click the **Delegated to Me** tile. Open the case — the delegation note and requesting pathologist are shown at the top of the right panel. Complete your review and either return the case or accept full ownership.

11. Case Search

Case Search (**Home** → **Search tile**) finds cases across your institution using any combination of filters. All active filters combine with AND logic.

11.1 Smart Identifier Box

What you type	Detection	Badge colour
S26-4200	Accession number (matches your lab format)	Purple
Smith, Jane or Jane Smith	Patient name	Green
123456 (numeric only)	MRN / Hospital ID	Cyan
abc123 (mixed)	All fields — searches simultaneously	Amber

11.2 Search Filters

Filter	Notes
Accession Date	Date window. Quick-pick: 7d, 30d, 90d, 1yr. Default is last 30 days.
Status	Grossed, Awaiting Micro, Finalizing, Completed. Multi-select.
Priority	Routine or STAT.
Flags	Click Browse. Multi-select. Flags are additive (OR logic within the flag filter).
Synoptic Protocol	Filter by CAP or institutional protocol. Click Browse for the full library.
Pathologist	Search by surname or hospital. Multiple pathologists can be selected.
Specimen Type	Multiple specimens allowed.
Diagnosis	Free-text terms. Multiple terms allowed. Press Enter or + to add.
SNOMED CT	Four axes: Morphology, Body Structure, Procedure, Specimen.
ICD Codes	Search across all active coding systems (ICD-10, ICD-11, ICD-O).

Tip: Save a "Daily STAT" search with a 7-day window and STAT priority for instant morning triage.

12. Internal Messaging

PathScribe's internal messaging system is for secure lab communication. Click the speech bubble icon in the navigation bar to open the messaging drawer.

12.1 Reading Messages

Messages are listed in the left panel. Unread messages are bold with a blue dot. Urgent messages appear highlighted in red. Click any conversation to read the full thread.

12.2 Composing

Click the compose icon at the bottom of the message list. Type a recipient name in the **To:** field. Enter subject and body. Toggle **Urgent** if required. Click **Send**.

12.3 Request Informal Review

To request an informal review from a colleague without transferring case ownership, open the case and use the **Request Review** option in the case header. Select a colleague, choose a review type (Informal Review, Consultation, Second Opinion, Clinical Observation), add an optional note, and send. The colleague receives a message with a direct link to the case.

■ **Note:** Request Informal Review does not transfer case ownership. It sends a message only. Use Case Delegation (Section 10) to formally transfer a case.

13. Voice Commands

PathScribe is fully voice-enabled — every action available via mouse or keyboard can be triggered by voice. Click the microphone icon to activate. The system is context-aware: the same phrase does different things on different screens.

Area	Example commands
Navigation	Open worklist · Open search · Open messages · Go home · Go back
Case navigation	Next case · Previous case · Open case [accession number]
Synoptic	Accept suggestion · Reject suggestion · Accept all suggestions · Next field · Save report
Finalisation	Finalise report · Submit for review
Messaging	Open messages · Compose message · Send message · Mark read
Search	Search cases · Clear filters · Export results
System	Open configuration · Sign out · Open voice settings

Best results: Use Chrome or Edge. Ensure your voice profile (EN-US or EN-GB) is set correctly in your profile settings. Speak clearly after the activation tone. PathScribe recognises 139 registered actions and learns unrecognised phrases permanently.

14. Your Profile

Click your name or initials badge (top right) to open the profile panel.

Option	Details
User Guide	Opens this document in a new browser tab.
Admin Guide	Opens the Admin Guide (visible to Admin and Pathologist-Admin roles only).
System Information	Application version, AI provider and model, API connectivity status, browser and OS details.
Copy to Clipboard	Generates a formatted support report. Name and ID are redacted — safe to share with PathScribe support.

15. Quick Reference

Action	How
Return to Home	Click the PathScribe by ForMedrix AI logo (top left)
Open Worklist	Home → Worklist tile · voice: "open worklist"
Open a case	Click any worklist row
Navigate specimens	Click specimen tabs A, B, C... in the right panel
View patient history	Patient History link in the left panel
Accept AI suggestion	Click tick (✓) next to the suggestion
Accept all suggestions	"Accept All" button at the top of the synoptic panel
Review protocol change	Protocol Change Review modal — appears automatically after microscopic is saved (v0.9.0)
View computational results	Click Computational tab in the synoptic panel
Order an additional test	+ Order additional test at the bottom of the Computational navigator
Preview report	Preview Report button in the case header
Finalise a case	Finalise button → 3-step gate → Pre-Finalisation Review → sign
Reorder synoptics before sign-out	Drag specimens in the Pre-Finalisation Review left pane (v0.9.0)
Exclude a synoptic from transmission	Click Exclude in the Pre-Finalisation Review left pane (v0.9.0)
Delegate a case	Delegate button in the case header
Request informal review	Request Review option in the case header
Claim a pool case	Pool Cases tile → click a pool case row → Claim
Search all cases	Home → Search tile
Send a message	Nav bar speech bubble → compose icon
Get help	Your initials badge (top right) → User Guide
Support report	Your initials badge → System Information → Copy to Clipboard

Appendix A — Colour & Visual Language Reference

This appendix explains the meaning of every colour, icon shape, and status indicator used across PathScribe.

A.1 Case Status Indicators (Worklist dots)

Colour	Meaning
Red — STAT / Urgent	Case flagged STAT or has a pending urgent action. Requires immediate attention.
Amber — Action Required	Needs user input: outstanding query, delegation pending, or a required field is missing.
Blue — In Progress	Actively being worked on (grossing, microscopy, or synoptic in draft).
Green — Complete / Signed Out	Report finalised and countersigned where required.
Grey — Awaiting Micro / Grossed	Specimen received and grossed; sections processing; no pathologist action yet required.
Purple — Delegated	Case delegated to another pathologist or in a review pool.

A.2 Case Flags

Colour	Flag class	Examples
Teal	Administrative	RUSH, Frozen Section, Tumour Board, MDT Discussion, QI Review
Cyan	Computational	IHC panels, molecular assays, BRAF, PD-L1, ER/PR/HER2
Amber	Quality / Review	Amended Report, Addendum Added, Correlation Required, Second Opinion
Green	Positive / Complete	AI coding applied, countersign received, research consent captured
Red	High-severity	Critical findings or urgent process flags

A.3 TAT Progress Bars

Colour	% of SLA consumed	Action
Green	< 70%	Well within the SLA. No intervention needed.
Amber	70–90%	Approaching the SLA limit. Monitor closely.
Red	> 90%	At or exceeding the SLA. Escalation or prioritisation required.

A.4 Severity Badges (Quality Tab)

Badge	Meaning
LOW (green)	Minor documentation issue or concordant variance. No immediate escalation required.

Badge	Meaning
MEDIUM (amber)	Moderate discordance (e.g. grade upgrade) or process deviation. Discuss at QI review.
HIGH (red)	Significant diagnostic discordance or patient-safety relevant amendment. Requires formal root-cause analysis.